

Reviews Evidence

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GTD practices (GTD) — · 950 reviews

All GTD-managed practices combined

ACCESS · 1.7 AVG

331 reviews

Access

This file describes a service that is hard to reach by phone,¹ hard to reach at the point of booking,² and often hard to use even after contact is finally made.³

Phone access is the most repeated failure.⁴ Reviews describe calls ringing unanswered for long periods,⁵ callers stuck at number one in the queue,⁶ busy lines and repeated redialling,⁷ and calls apparently being cut off or ignored.⁸

The 8am model is a major source of frustration.⁹ People describe being told to ring at 8am,¹⁰ then getting through only after the day's appointments have already gone,¹¹ or being told to try again the next morning and repeat the same process.¹²

The problem is not just delay; it is scarcity.¹³ Same-day appointments are described as nearly impossible to get,¹⁴ pre-bookable appointments are often described as unavailable or very limited,¹⁵ and some people report waiting weeks for routine appointments.¹⁶

Patients repeatedly say the system is especially bad if you work,¹⁷ care for someone else,¹⁸ or need help for a child or vulnerable adult.¹⁹ Several reviews describe people missing work, lunch breaks, or other responsibilities just to stay on hold or try to secure a slot.²⁰

A recurring complaint is that reception or admin staff act as gatekeepers rather than facilitators.²¹ Reviews say patients are blocked before they can speak to a GP,²² non-clinical staff make or relay clinical decisions,²³ and access is refused or diverted without clear ownership.²⁴

Reception conduct is itself a major theme.²⁵ Reviews repeatedly describe staff as rude,²⁶ dismissive,²⁷ patronising,²⁸ unhelpful,²⁹ and lacking empathy when patients are already stressed or unwell.³⁰

A second pattern is being redirected away from GP care.³¹ Patients report being told to go to a pharmacist,³² to call 111,³³ to use a walk-in centre,³⁴ or to attend A&E,³⁵ sometimes after already being told by those same services to contact the GP.³⁶

That creates a loop rather than a pathway.³⁷ Reviews describe people being bounced between GP, hospital, 111, pharmacy, and walk-in services,³⁸ with nobody clearly taking responsibility for resolving the issue.³⁹

Digital systems are presented less as convenience and more as another barrier.⁴⁰ Reviews describe online-only or online-first booking,⁴¹ confusing forms or websites,⁴² the PATCHS system as difficult or obstructive,⁴³ and patients being told to “go online” even when phone contact failed.⁴⁴

The digital route is not described as reliably open either.⁴⁵ Reviews say forms close early,⁴⁶ there is “never any appointments” after submitting online,⁴⁷ and texted instructions for online access can be unclear or incomplete.⁴⁸

Several reviews say admin handling is unreliable even when contact is made.⁴⁹ Appointments are described as changed without notice,⁵⁰ cancelled on the day,⁵¹ entered wrongly,⁵² or simply missing from the system when patients arrive.⁵³

That unreliability extends to follow-through.⁵⁴ Reviews describe promised callbacks not happening,⁵⁵ letters and referrals being lost,⁵⁶ results not being communicated on time,⁵⁷ and patients having to chase the same issue repeatedly themselves.⁵⁸

Some reviews describe people waiting a long time only to receive no meaningful care.⁵⁹ Patients report sitting in waiting rooms for 35 to 50 minutes before being told an appointment was cancelled or elsewhere,⁶⁰ and telephone appointments that never happened despite being booked.⁶¹

Clinical continuity is weak in the source material.⁶² Reviews mention only one effective GP at a practice,⁶³ reliance on locums,⁶⁴ frequent use of non-GP clinicians where patients expected a doctor,⁶⁵ and long gaps without face-to-face GP contact.⁶⁶

Prescription management is another repeated failure mode.⁶⁷ Reviews describe repeat prescriptions running late,⁶⁸ medication being withheld pending reviews that were themselves hard to book,⁶⁹ patients being left without important medication for days,⁷⁰ and recurring monthly stress around basic prescription processing.⁷¹

The overall complaint is not just poor manners or long waits.⁷² The reviews describe a service patients experience as obstructive,⁷³ fragmented,⁷⁴ error-prone,⁷⁵ and unsafe at the edges where admin failure turns into delayed or missed care.⁷⁶

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RECEPTION AND FRONT DESK · 1.9 AVG

145 reviews

Front desk

This file is less about the abstract problem of access and more about what patients say happens when they actually meet the practice at the front desk or on the phone.¹ The repeated complaint is not just delay but antagonism.²

Reception is described as rude,³ dismissive,⁴ patronising,⁵ aggressive,⁶ and sometimes openly humiliating.⁷ Several reviews say patients dread dealing with reception before they even try to ask for help.⁸

The front desk is repeatedly described as acting like a barrier rather than a route into care.⁹ Patients say staff decide who is worth helping,¹⁰ enforce rules inflexibly or selectively,¹¹ and make people feel they are asking for a favour rather than using a service.¹²

A sharper theme here is non-clinical overreach.¹³ Reviews say receptionists speak as if they are making clinical decisions,¹⁴ decide a child does not need a GP,¹⁵ redirect people to pharmacists or other services without clear ownership,¹⁶ and sometimes appear to overrule or block direct contact with a clinician.¹⁷

This file also adds a stronger picture of clerical unreliability at desk level.¹⁸ Patients describe reception booking the wrong appointment,¹⁹ giving the wrong time or location,²⁰ cancelling or changing appointments without notice,²¹ failing to check patients in properly,²² and then treating the resulting disruption as the patient's problem.²³

Another distinctive thread is that patients often feel front-desk staff do not listen even to basic facts.²⁴ Reviews describe being cut off mid-explanation,²⁵ told to go online before finishing the sentence,²⁶ denied messages or callback requests,²⁷ and having obvious context such as childcare, work, disability, or prior instructions brushed aside.²⁸

Some reviews go further and describe deception or evasion.²⁹ Patients say staff denied previous conversations,³⁰ claimed forms or letters had been processed when they had not,³¹ said calls or messages would be passed on when nothing happened,³² and in a few cases appeared to ignore ringing phones deliberately.³³

The file also gives stronger evidence of front-desk conduct spilling into dignity and confidentiality issues.³⁴ Patients describe being challenged loudly in public,³⁵ embarrassed at reception,³⁶ made to explain private medical matters within earshot of others,³⁷ and spoken to in ways that would be unacceptable in any basic customer-facing role, never mind healthcare.³⁸

A recurring complaint is that reception behaviour becomes even worse when the patient is vulnerable.³⁹ Parents seeking help for babies or young children describe being brushed off or handled harshly,⁴⁰ people with mental-health problems describe being patronised or discriminated against,⁴¹ and unwell or distressed patients describe staff reacting with indifference, contempt, or impatience.⁴²

This file also makes the emotional tone of the interaction much clearer than the earlier reports.⁴³ Reviews describe staff smirking while someone is in pain,⁴⁴ giggling after dismissing an elderly patient,⁴⁵ talking about lunches or making tea while callers wait,⁴⁶ and generally behaving as if the patient's problem is an interruption to their day rather than the reason the service exists.⁴⁷

Complaints and escalation do not appear to repair that relationship.⁴⁸ Reviews describe managers being unavailable or shielding staff,⁴⁹ feedback forms and complaint routes leading nowhere,⁵⁰ and patients concluding that formal escalation is the only language the practice will understand.⁵¹

What this file adds, beyond the broader access and admin failures already captured elsewhere, is a more direct account of interpersonal breakdown at the point of contact.⁵² The front desk is not only described as inefficient.⁵³ It is described as adversarial,⁵⁴ clerically unreliable,⁵⁵ casually disrespectful,⁵⁶ and in some cases unsafe because reception behaviour directly alters whether care happens at all.⁵⁷

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CLINICAL CARE · 2.4 AVG

106 reviews

Clinical care

This file is not mainly about booking friction. It is about patients saying that, once they do reach the service, the clinical side can still feel unsafe, superficial, or simply absent.¹²

A repeated complaint is that patients are not properly assessed before decisions are made.³⁴ Reviews describe conditions being handled over the phone in a couple of minutes,⁵ people being told what they do or do not need without examination,⁶ and obvious clinical uncertainty being left unresolved rather than investigated.⁷

Several reviews go further and describe outright misdiagnosis or wrong clinical judgement.⁸⁹ Patients say they were given the wrong diagnosis over the phone,¹⁰ their symptoms were reduced to painkillers instead of being investigated,¹¹ or they were reassured that something serious was "a mystery" and then simply not followed up.¹²

The strongest clinical-safety complaints are about records and medication being attached to the wrong person.¹³ One patient says a nurse insisted they were taking thyroxine when they had never been prescribed it,¹⁴ and the same practice had previously told that patient they were twelve weeks pregnant when they were not.¹⁵

Medication complaints here are not just about delays. They include unsafe prescribing and weak review.¹⁶ One review says the wrong medication was prescribed three times in a row,¹⁷ another says birth-control pills were prescribed over the phone despite a missed period,¹⁸ and another says a baby's immunisation advice was wrong enough that the child ended up in hospital.¹⁹

There is also a recurring complaint that staff prescribe or repeat treatment without properly reconsidering whether it is working.²⁰ One parent says their child's rash cream was reissued after the receptionist spoke to the GP, even though the cream had already failed,²¹ and another patient says they were sent a prescription before anyone explained what it was for or what the results actually meant.²²

A second pattern is substitution of proper GP assessment with thinner forms of contact.²³ Patients describe being sent to pharmacists instead of GPs,²⁴ being handled by nurses when they believed the issue clearly needed a doctor,²⁵ or being pushed to walk-in centres and A&E for problems they believed needed continuing primary-care management.²⁶

That matters because the file repeatedly links those substitutions to worse outcomes, not just inconvenience.²⁷ Reviews say children and vulnerable patients were left without review,²⁸ chronic problems escalated until hospital attendance was needed,²⁹ and some people now avoid the surgery altogether because they expect to be dismissed or mismanaged.³⁰

Children's care is one of the clearer sub-themes in this file.³¹ Parents describe repeated cancellation of immunisation clinics,³² rushed injections with poor handling of distressed babies,³³ babies and young children being denied or delayed GP review,³⁴ and parents being told to keep re-attending only once the child gets worse again.³⁵

Mental-health care also appears as an area where the service feels particularly brittle.³⁶ Reviews describe staff who did not seem to understand mental-health urgency,³⁷ patients feeling dismissed or unheard when they were already struggling,³⁸ and a sense that emotional distress is treated as nuisance rather than clinical need.³⁹

Test results and follow-up are another clinical weak point in this file.⁴⁰ Reviews mention long delays in blood results,⁴¹ being told there was "a problem" with a child's bloods and then being left to wait weeks,⁴² and patients having to chase results that should have triggered timely review.⁴³

Continuity is weak in a way that patients think affects judgement.⁴⁴ Reviews mention constant staff turnover,⁴⁵ reliance on locums who only have minutes to read the notes,⁴⁶ and doctors changing so often that patients feel no one is really carrying the case.⁴⁷

The broad picture is of clinical care that is often experienced as cursory,⁴⁸ poorly reasoned,⁴⁹ badly followed through,⁵⁰ and sometimes unsafe in concrete ways rather than merely frustrating ones.⁵¹

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ADMIN AND REFERRALS · 1.4 AVG

66 reviews

Administration, referrals, records, and follow-through

This file is not mainly about the difficulty of getting through the door; the earlier report already covers that.¹ What stands out here is what happens after contact: paperwork goes missing,² referrals are not sent or are sent wrongly,³ prescriptions are delayed or mishandled,⁴ appointments and tests are cancelled or mis-recorded,⁵ and patients are forced to do the chasing, clarifying, and repair work themselves.⁶

Document handling looks repeatedly unreliable.⁷ Hospital letters are described as lost even after being handed in physically and resent by photo,⁸ change-of-prescription letters are said to disappear from the system,⁹ forms for insurance, proof of address, and other routine admin are described as taking weeks or months for work patients see as a five-minute job,¹⁰ and records transfers are described as slow enough to disrupt care or external processes.¹¹

Referral handling is one of the strongest themes in this file.¹² Reviews describe referrals that were promised but never followed up,¹³ referrals written so inaccurately that the receiving service refused or altered the appointment,¹⁴ referrals sent to the wrong clinician,¹⁵ referrals delayed by months,¹⁶ and patients being told to chase hospital departments themselves for work they believed the practice should complete.¹⁷

The source also contains repeated claims that internal admin failure directly alters clinical care.¹⁸ Reviews describe staff having the wrong patient open on screen,¹⁹ patients being told they were pregnant when they were not,²⁰ blood results being relayed incorrectly,²¹ reception staff giving what patients understood as clinical decisions or triage outcomes,²² and non-clinical staff blocking, redirecting, or narrowing care in ways patients considered unsafe.²³

Prescription handling is not just slow here; it is disorderly.²⁴ Reviews describe repeat requests going missing,²⁵ scripts arriving on the twenty-eighth day month after month,²⁶ weekly prescriptions failing weekly,²⁷ wrong medication being issued repeatedly,²⁸ long-term medicines being altered or stopped without clear communication,²⁹ and patients being left without essential medication for days, weeks, or even months while trying to get basic corrections made.³⁰

Appointments, tests, and callbacks are presented as administratively fragile.³¹ Reviews describe telephone reviews being cancelled minutes before they were due,³² immunisation clinics repeatedly cancelled the day before,³³ blood tests cancelled more than once because the nurse was not available,³⁴ scans cancelled with little notice and never rearranged,³⁵ booked appointments apparently disappearing or being entered with the wrong date,³⁶ and promised follow-up calls simply not happening.³⁷

Another pattern here is duplication, repetition, and circular process.³⁸ Patients describe filling in the same form multiple times,³⁹ being told to submit another PATCHS request instead of having the issue resolved,⁴⁰ being asked to book a GP appointment purely so the GP can re-refer them back to a nurse for tests already requested elsewhere,⁴¹ and being bounced between GP, hospital, orthopaedics, consultants' secretaries, 111, and walk-in services because nobody takes ownership of the next action.⁴²

Complaint handling is itself part of the problem.⁴³ Reviews describe requests for managers being blocked,⁴⁴ feedback forms producing no response,⁴⁵ complaints promised but not acknowledged,⁴⁶ formal letters requested for appeals or escalation not arriving,⁴⁷ and patients concluding that the practical effect of the system is to make challenge, correction, and accountability harder than they should be.⁴⁸

There are also explicit confidentiality and records-governance concerns that go beyond ordinary rudeness.⁴⁹ Reviews describe someone else's personal information being posted out to the wrong patient,⁵⁰ staff reading another patient's records over the phone,⁵¹ private medical discussions taking place in open or inappropriate

settings,⁵² and staff being accused of mishandling, delaying, or failing to update records in ways that affect both trust and care.⁵³

The cumulative complaint is that administration is not a background problem here.⁵⁴ In these reviews it is the thing that distorts care pathways,⁵⁵ delays treatment,⁵⁶ breaks continuity,⁵⁷ creates avoidable extra appointments across the NHS,⁵⁸ and leaves patients feeling that basic clerical competence cannot be relied on even before the clinical question is reached.⁵⁹

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MEDICINES AND TESTS · 1.5 AVG

82 reviews

Medicines and tests

This file is less about first contact than the earlier reports. It is about what happens once medication, monitoring, bloods, scans, or results are supposed to move through the system.¹² The strongest pattern is not a single delay but a recurring sense that prescribing and test follow-up are unreliable enough to become clinical risks in their own right.³⁴

Repeat prescribing is the dominant problem.⁵⁶ Reviews describe medication requests sitting unsigned for days or weeks,⁷ being delayed month after month,⁸ being rejected without clear warning,⁹ or being processed so late that patients ration tablets, go without, buy medication privately, or seek emergency cover from 111.¹⁰

The complaints are not only about slowness. They are also about correctness.¹¹¹² Patients describe prescriptions arriving wrong,¹³ incomplete,¹⁴ missing items that should have been routine,¹⁵ being stopped without adequate explanation,¹⁶ or being repeatedly mishandled even when the same medicines had been stable for years.¹⁷

Several reviews push this beyond inconvenience into obvious safety territory.¹⁸ One patient says they were left without anticonvulsants for five days,¹⁹ another says a child's potentially life-saving medicine was still not signed after nearly a week,²⁰ another describes going "cold Turkey" because repeat medication was so unreliable,²¹ and others describe pain relief or chronic-condition medication being withheld while the patient is told to chase reviews they cannot actually secure.²²

A related theme is that medication review has become a bottleneck rather than a safeguard.²³²⁴ Patients describe being told no prescription without a review,²⁵ then being unable to get the review appointment,²⁶ waiting weeks for it,²⁷ having it cancelled at short notice,²⁸ or turning up to find the wrong type of appointment had been booked and no one there could authorise the medicine.²⁹

The file also contains repeated complaints about basic medication reconciliation and record accuracy.³⁰ Patients say staff insisted they were on medicines they had never been prescribed,³¹ treated them as pregnant when they were not,³² rejected repeat requests despite those medicines already being on the system,³³ or attached the wrong patient record to the consultation altogether.³⁴

Another strong pattern is broken handoff between the surgery, the doctor, and the pharmacy.³⁵³⁶ Reviews repeatedly describe patients being bounced between reception and chemist,³⁷ being told the prescription was sent when it was not,³⁸ being told four items were issued when only one was issued,³⁹ or being forced to keep rechecking because nobody seemed able to say where the request actually was.⁴⁰

Tests are the second major theme.⁴¹ Blood tests, urine tests, scans, and sample handling are all described as unreliable.⁴² Reviews mention cancelled blood tests,⁴³ blood-test appointments that were hard to obtain even

when clinically needed,⁴⁴ sample pots being wrong,⁴⁵ samples being misplaced,⁴⁶ tests apparently lost,⁴⁷ and patients waiting in pain only to be told to rebook.⁴⁸

The problem continues after the test is done.⁴⁹ Results are described as slow,⁵⁰ missing,⁵¹ contradictory,⁵² or poorly communicated.⁵³ Patients say they were told there was a problem and then given a very different story later,⁵⁴ waited weeks or months for results,⁵⁵ were told to "check online" when that did not answer the issue,⁵⁶ or were sent medication before anyone had properly explained what the result meant or what diagnosis was being treated.⁵⁷

What stands out here is not just delay but weak clinical ownership of follow-up.⁵⁸ Patients describe abnormal or overdue monitoring not triggering timely action,⁶⁰ bloods being discussed by non-clinical staff in misleading ways,⁶¹ and clinically significant next steps, like scans, repeat bloods, medication changes, or explanation of side effects, being left for the patient to chase manually.⁶²

There are also complaints that prescribing decisions are sometimes being made, blocked, or filtered by reception rather than by a clinician in any meaningful way.⁶³ Reviews say receptionists refused prescription requests on process grounds,⁶⁵ acted as gatekeepers over medication changes,⁶⁶ or appeared to be making quasi-clinical decisions about urgency, suitability, and what type of script counted as appropriate.⁶⁷

The cumulative picture is that medicines and tests are not experienced here as dependable background processes.⁶⁸ They are experienced as fragile chains with multiple points of failure: the request may not be logged,⁷⁰ the doctor may not sign it in time,⁷¹ the pharmacy may not receive the right items,⁷² the monitoring review may not be bookable,⁷³ the blood test may be cancelled or mishandled,⁷⁴ and the result may still not lead to a clear explanation or action plan.⁷⁵

There are a few positive reviews in the file, mostly about individual blood-test encounters being punctual and professional, and one more recent account of a doctor carefully explaining results.⁷⁶ But those are outnumbered by the reviews describing medication and test systems as inconsistent, opaque, and at times unsafe.⁷⁸

► Show Cited Reviews

ON-SITE EXPERIENCE · 1.3 AVG

20 reviews

On-site attendance is repeatedly described as effort without care: people make the trip, arrive on time, sit waiting, and still leave untreated, cancelled, rebooked, or redirected.¹²³

Patients describe being kept in waiting rooms for long periods before being told the appointment has been cancelled or cannot go ahead.⁴⁵⁶

Several reviews describe delays as especially harmful because attendance itself was difficult: people had rearranged work, brought distressed children, attended while in pain, or arrived with limited mobility.⁷⁸⁹

The waiting-room experience is described as poorly managed rather than merely busy.¹¹¹

People report waiting rooms that appear empty or lightly occupied while appointments still do not happen, which sharpens the sense that the blockage is organisational rather than unavoidable demand.¹⁴¹

Some accounts say the practice allowed patients to wait for 30 to 60 minutes, or longer, without telling them there was already a problem with the appointment.¹⁶¹

A recurring complaint is that patients are allowed to arrive, queue, check in, or fill paperwork, only to learn later that treatment is unavailable, the clinician is absent, or the slot has effectively been lost.¹⁹²

Check-in and front-of-house processes are described as error-prone and confusing.²²²³

New patients say they were not told they needed to check in, were sent to the wrong waiting area, or were then treated as though the failure was theirs.²⁴²⁵

Broken or badly managed self-check-in also appears as a practical barrier that turns a booked visit into a missed one.²⁶

Several reviews describe staff responding to delay or confusion with blame rather than repair.²⁷²⁸²⁹

Patients say they were told that if they did not want to wait they could cancel, that they had "cancelled" appointments they say they were actively waiting for, or that they had lost the slot after being given the wrong details.³⁰³¹³²

The in-person environment itself is sometimes described as undignified or inappropriate for care.³³³⁴

One review describes being made to discuss the reason for an appointment outside in the street because there was no waiting room access.³⁵

Another describes examinations being carried out in a baby-changing room that also functioned as a waiting area, described as cramped and unsuitable.³⁶

Privacy, dignity, and basic patient handling therefore appear as part of the on-site problem, not just speed.³⁷³⁸

There are also repeated complaints that physical attendance does not create any practical route past administrative blockage.³⁹⁴⁰

Even when patients or relatives come in person, reception is described as refusing to pass messages through, refusing help, or sending people away with no useful action.⁴¹⁴²

Several reviews say the visit ends with the patient being told to rebook, return later, contact the GP again, or go elsewhere.⁴³⁴⁴⁴⁵

Some of the strongest accounts involve missed or mishandled on-site diagnostics.⁴⁶⁴⁷

Patients report blood tests being booked and then cancelled after arrival, ultrasound appointments where no one ever called them in, and repeat blood-test cancellations linked to absent staff.⁴⁸⁴⁹⁵⁰

That matters because the complaint is not just delay in the abstract; it is pain, worry, work disruption, and repeated attendance for care that never actually materialises.⁵¹⁵²⁵³

There is also a narrower but important thread about visible mismatch between apparent capacity and actual help.⁵⁴⁵⁵

Reviews describe empty waiting rooms, visible staff, or phones ringing unanswered while patients are told nothing is available.⁵⁶⁵⁷

This makes the on-site experience feel arbitrary: patients can be physically present in the building and still unable to reach anyone empowered to solve the problem.⁵⁸⁵⁹

A smaller number of reviews do add nuance: one patient said PATCHS authorisation worked quickly and the face-to-face reception interaction was not unpleasant, even though telephone access and appointment delay remained poor.⁶⁰

But the overall pattern in this file is that attending in person is often described as a dead end rather than a safeguard against system failure.⁶¹⁶²

► [Show Cited Reviews](#)

2 reviews

Management and change

This file is about failure being attributed to how practices are run, not just to day-to-day pressure.¹ Reviewers repeatedly say standards worsened after a takeover, a permanent contract, a management change, or the introduction of a new system.²

The strongest thread is that change is experienced as disruption without protection for existing patients.³ People describe booked appointments not being migrated onto replacement systems,⁴ long-standing routes to contact being withdrawn,⁵ working arrangements changing without notice,⁶ and new digital processes arriving before they reliably worked.⁷

Several reviews say management change did not just fail to fix old problems but actively made them worse.⁸ Patients describe previously simple prescription requests becoming slower and riskier after migration to new apps or identity checks,⁹ online appointment systems becoming narrower or time-limited,¹⁰ and formerly available options like walk-in, phone, weekend, or baby-priority access being reduced or scrapped.¹¹

This file adds a clearer picture of botched transition management.¹² Reviewers describe old appointments being dumped rather than transferred,¹³ patients being told to start again in the usual 8am scramble,¹⁴ immunisation clinics or nurse sessions being repeatedly cancelled under new arrangements,¹⁵ and administrative promises about forms, callbacks, or medication reviews collapsing during the handover period.¹⁶

A distinct complaint here is that practices appear to measure success by clearing systems rather than resolving care.¹⁷ One reviewer describes a PATCHS request being marked complete without diagnosis, treatment, or a usable onward booking.¹⁸ Others describe requests formally processed but not actually actioned, such as rejected repeats, undelivered referral letters, or unresolved admin forms.¹⁹

Leadership itself is a direct target in this file.²⁰ Reviews call managers invisible,²¹ unavailable,²² unwilling to intervene,²³ or responsible for allowing poor staff behaviour and broken systems to continue for months or years.²⁴ Where change is mentioned explicitly, patients often say new management promised improvement but left the same practical failures in place.²⁵

Another theme is strategic confusion.²⁶ Reviewers describe conflicting instructions across phone lines, websites, text messages, and reception,²⁷ being told to use systems that were closed, inaccessible, or not configured for registered patients,²⁸ and being moved between GP, pharmacist, physiotherapy, walk-in, 111, or hospital pathways without clear clinical ownership.²⁹

This file also adds stronger evidence that management decisions are narrowing the definition of service.³⁰ Patients say requests are being channelled into fixed digital windows,³¹ sick notes and admin requests are being time-limited like appointments,³² non-doctor staff are increasingly substituted for requested GP review,³³ and routine matters are redesigned around practice convenience rather than patient use.³⁴

A repeated point is that these are not described as isolated bad interactions.³⁵ Reviewers explicitly compare the current service with how the same practice worked before,³⁶ say decline has taken place over several years,³⁷ and link the drop in standards to concrete managerial choices such as changing contracts, removing clinics, replacing systems, or failing to supervise staff.³⁸

There is also a more explicit accountability problem here than in the earlier files.³⁹ Patients describe complaints, feedback forms, and escalation routes that produce no answer,⁴⁰ requests for formal letters being ignored,⁴¹ and

managers appearing easier to mention in reviews than to reach in practice.⁴² Some reviewers move straight from failed complaint routes to changing surgery or escalating outside the practice.⁴³

Where this file adds something genuinely new is in the pattern of organisational self-disruption.⁴⁴ The complaint is not only that access is hard, admin is messy, or reception is rude, all of which appear elsewhere.⁴⁵ It is that restructures, system swaps, and management interventions are repeatedly described as the thing that turned strain into dysfunction, because the practice changed its processes without preserving continuity, clarity, or responsibility.⁴⁶

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POSITIVE FEEDBACK · 4.8 AVG

18 reviews

Positive reviews read differently from the other sets. Sorting by positivity makes every operational area look better than it does in the issue-led files,¹ but many of these reviews are not really about a discrete system failure or service line at all.² They are often simple thank-you notes,³ broad endorsements of named staff,⁴ or grateful accounts of being helped when the patient felt worried, vulnerable, or stuck.⁵ As a result, this set pools most strongly around tone, kindness, reassurance, and gratitude,⁶ rather than around a neat spread of operational areas.⁷

The strongest positive signal is not “the system works perfectly”.⁸ It is that individual staff members are repeatedly described as kind, calm, helpful, respectful, reassuring, and willing to go beyond the minimum.⁹ Reception staff are thanked for listening, sorting things out, making appointments, chasing problems, and treating people warmly under pressure.¹⁰ Nurses are praised for gentleness, technical skill, and helping anxious patients through blood tests, injections, smear tests, and other procedures.¹¹ Doctors and advanced clinicians are most often praised for listening properly, explaining clearly, following up, being thorough, and taking concerns seriously.¹²

A lot of the positivity is highly personalised.¹³ Reviews repeatedly name individual staff members as the reason for the rating: Lisa,¹⁴ Raghia,¹⁵ Becki,¹⁶ Jess,¹⁷ Emma and Debbie,¹⁸ Jane,¹⁹ Precious,²⁰ Fasih,²¹ Dr Metcalfe,²² 14 14 14 14 14 3
That matters because it suggests the positive experience is often attached to particular¹⁴ not just to abstract confidence in the practice¹⁴ 5

Clinical praise is usually concrete when it appears.²⁹ Patients value being listened to,³⁰ being taken seriously,³¹ getting the right diagnosis quickly,³² having investigations arranged,³³ receiving careful explanations,³⁴ and being treated with compassion during stressful or painful care.³⁵ Several reviews describe staff creating calm around anxiety-provoking procedures such as blood tests, injections, smear tests, and dental extraction.³⁶ Others praise continuity and follow-up, including calls back, checking progress, and keeping on top of ongoing problems.³⁷

There is some genuine positive evidence on access and responsiveness,³⁸ but it is mixed rather than universal.³⁹ Some patients say they got through quickly,⁴⁰ got same-day help,⁴¹ found PATCHS easy to use,⁴² or were offered appointments that worked for work or family life.⁴³ Some also praise proactive booking behaviour, including call-backs about cancellations or staff finding another route to care on the day.⁴⁴ But even within positive reviews, people still mention waits on the phone,⁴⁵ delays for routine appointments,⁴⁶ and the general scarcity of appointments as part of the background normality of primary care.⁴⁷ So this file softens the access picture more than rather than universal.³⁹ 0

What the positive set adds, more than the earlier reports, is evidence about emotional temperature.⁴⁹ Patients repeatedly describe feeling calmed down,⁵⁰ reassured,⁵¹ comfortable,⁵² at ease,⁵³ welcomed,⁵⁴ and looked after with kindness and respect.⁵⁵ Parents especially value staff who respond well to children,⁵⁶ and anxious patients value staff who keep procedures gentle, explain what is happening, and make them feel safe.⁵⁷ That is a real service quality signal in its own right,⁵⁸ even when the review does not say much about the wider machinery feeling calmed down,⁵⁰ 0

The positive reviews also contain a noticeable defence of staff against other reviewers.⁶⁰ Some explicitly argue that receptionists get unfair abuse,⁶¹ that demand exceeds capacity,⁶² that patients should be realistic,⁶³ and that NHS pressures or funding constraints should be taken into account.⁶⁴ A few positive reviews are almost counter-arguments to the negative corpus, insisting that the surgery has improved,⁶⁵ that the reviewer has “never had a problem”,⁶⁶ or that criticisms are exaggerated compared with the care they themselves received.⁶⁷ That does not erase the problems in the other files,⁶⁸ but it does show a substantial cluster of patients whose frame is tolerant, appreciative, and staff-protective.⁶⁹

Overall, this set is less useful for diagnosing a single operational failure pattern than the themed complaint files.⁷⁰ Its value is different.⁷¹ It shows which parts of the service generate gratitude,⁷² which behaviours patients remember most,⁷³ and which staff roles most often convert a difficult interaction into a positive one.⁷⁴ The recurring positive language centres on kindness,⁷⁵ listening,⁷⁶ reassurance,⁷⁷ professionalism,⁷⁸ efficiency when it matters,⁷⁹ and staff making extra effort inside value is different.⁷¹ 0

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OTHER · 2.4 AVG

180 reviews

Residual / uncategorised reviews

This residual set changes the picture less by adding a new operational theme than by showing what is left over once the clearer categories have been pulled out.¹ Most entries here are too short to support a detailed issue map.² They read less like structured complaints about one pathway and more like compressed verdicts on the service as a whole.³

That matters in itself.⁴ After the access, admin, clinical, medicines, front-desk, on-site, management, and positive sets are separated out, the remaining material still leans heavily negative.⁵ Even without detail, patients are still using blanket language of failure,⁶ collapse,⁷ uselessness,⁸ disrespect,⁹ and wasted effort.¹⁰

A first pattern is pure service repudiation.¹¹ Several reviews offer almost no narrative at all, only a verdict that the surgery is a joke,¹² a shambles,¹³ useless,¹⁴ very poor,¹⁵ demoralising,¹⁶ or a waste of time.¹⁷ These do not tell us much about process,¹⁸ but they do show that some patient experiences are being remembered not as one bad incident but as total service failure.¹⁹

A second pattern is that the overlap with earlier themes remains visible even in this leftover file.²⁰ Capacity pressure still appears in miniature through claims that services are always full,²¹ that vital medicine is delayed for over a week,²² and that blood tests were done for the wrong thing.²³ Those points are already covered elsewhere,²⁴ but their presence here shows how often the same failures collapse into a short general condemnation rather than a detailed review.²⁵

The front-desk and interpersonal layer also still shows through.²⁶ One review describes staff as disrespectful,²⁷ says even a request for drinking water was ignored,²⁸ and says the patient was told to buy water elsewhere.²⁹ Another says the attitude was disgusting.³⁰ Another describes neglect, humiliation, and staff making life harder rather than helping.³¹ That adds some new colour to the earlier front-desk material because the complaint here is not just rudeness in passing;³² it is a sense of basic disregard for dignity and welfare on site.³³

There is also a small but useful signal about safety and competence.³⁴ One patient says bloods were done for the wrong thing,³⁵ and ties that to an attempted iron-related follow-up despite a known anaphylactic allergy.³⁶ Another reports waiting more than a week for vital medicine.³⁷ These are too sparse to form a new category of their own,³⁸ but they reinforce the broader picture from the clinical and medicines files that mistakes in handling and follow-through are not being experienced as trivial admin slips.³⁹

This file also contains a few almost content-free attacks on work ethic or named clinicians.⁴⁰ “Shift Dodgers”⁴¹ and “Dr Singh 🙄”⁴² are evidentially thin,⁴³ but they still matter as sentiment.⁴⁴ They show a residue of distrust that is strong enough to generate a public negative review even when the reviewer gives almost no explanation.⁴⁵

The positive residue is much thinner and much less informative than the negative residue.⁴⁶ The few favourable entries are extremely brief: “Good”,⁴⁷ “Prompt, pleasant run practice”,⁴⁹ and one short note that a doctor is knowledgeable and treats patients nicely.⁵⁰ Compared with the dedicated positive set, these do not build a rival picture of service strengths.⁵¹ They mostly confirm that short-form praise tends to be generic unless the reviewer is motivated enough to describe a specific helpful interaction.⁵²

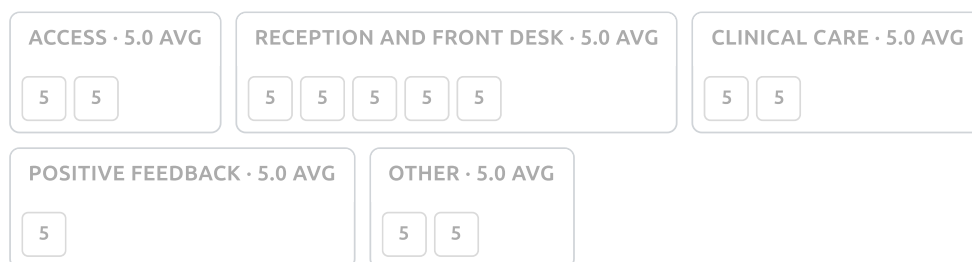
Taken together, this uncategorised remainder works best as an ending note to the review corpus rather than a major theme in its own right.⁵³ It shows what the structured categories do not fully capture:⁵⁴ a layer of compressed public judgement,⁵⁵ emotionally charged but low-detail dissatisfaction,⁵⁶ and a residual sense that for some patients the service fails too broadly to be reduced to one broken step.⁵⁷

Across the whole set of review summaries, the same overall outcome keeps resurfacing.⁵⁸ Patients are not mainly describing isolated inconvenience.⁵⁹ They describe repeated friction across contact, booking, front-desk handling, admin follow-through, clinical decision-making, medicines, tests, and the in-person experience.⁶⁰ Positive reviews show that individual staff often create reassurance, kindness, and competent care inside that system,⁶¹ but the negative material is more cumulative.⁶² It points to a service where trust is damaged not by one single fault line but by repeated small failures that stack together into delay,⁶³ confusion,⁶⁴ disrespect,⁶⁵ and, at the sharper edge, missed or unsafe care.⁶⁶

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Other fully reviewed (good) — · 12 reviews

Non-GTD practices with full reviews, included by positive reviews



Other fully reviewed (poor) — · 11 reviews

Non-GTD practices with full reviews, included by negative reviews